Dacomitinib (Vizimpro®)

Dacomitinib is a targeted therapy drug. It is used to treat non-small cell lung cancer (NSCLC). It may sometimes be used to treat other cancers.

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Dacomitinib (Vizimpro®)

Dacomitinib (Vizimpro®) is a type of drug called a tyrosine kinase inhibitor (TKI) and belongs to a group of targeted therapy drugs called cancer growth inhibitors. It is best to read the information about this drug with our general information about targeted therapy drugs and the type of cancer you have.

Kinases are proteins in the body that control how the cells grow and divide. This drug blocks the proteins (kinases) from sending signals to cancer cells to grow. Blocking the signals can cause the cancer cells to die. This can help to stop cancer from growing or slow it down.

Dacomitinib blocks (inhibits) a protein found in cells called epidermal growth factor receptor (EGFR). Sometimes a change happens in the EGFR which can help cancer cells to grow and spread. This drug can help control cancer by blocking the EGFR that has this change.

You will only have dacomitinib if the cancer cells have EGFR changes. Usually, a sample of cancer cells are taken during surgery or a biopsy and are tested. Sometimes, the change can be found in a blood test.

Your cancer doctor will talk to you about this treatment and its possible side effects before you agree (consent) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the electronic Medicines Compendium (eMC) website, which has patient information leaflets (PIL) for individual drugs.

How dacomitinib is given

Dacomitinib comes as tablets you take at home.

During treatment, you usually see a:

- cancer doctor
- cancer nurse or specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During treatment, a nurse or person trained to take blood (phlebotomist) will take regular blood samples from you. This is to check that it is safe for you to keep taking your treatment.

You will speak to a doctor, nurse or pharmacist regularly during treatment. They will ask how you have been feeling and check your blood results are okay.

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Your course of treatment

You usually take dacomitinib for as long as it is working for you and any side effects can be managed. Do not stop taking it without talking to your doctor.

Taking dacomitinib tablets

Your cancer team will give you the tablets to take home. Always take them exactly as explained. This is important to make sure they work as well as possible for you. You may be given tablets of different strengths.

You usually take dacomitinib once a day. Try to take your dose at the same time every day. Swallow the tablets whole with a glass of water. You can take the tablets with or without food.

If you forget to take a dose, do not take a double dose to make up for it. Just take the next dose at the usual time the following day.

Tell your doctor, nurse or pharmacist if you take medicines to control stomach acid such as drugs to treat heartburn, ulcers or indigestion. These types of drugs can affect dacomitinib. Your doctor may change these drugs to a different type of treatment. Or they may advise you to take your dacomitinib at least 2 hours before, or 10 hours after, these drugs.

Other things to remember about your capsules/tablets:

- Wash your hands after taking your tablets.
- Other people should avoid direct contact with the drugs.
- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, somewhere children cannot see or reach them.
- If you are sick just after taking the tablets, contact the hospital. Do not take another dose.
- If your treatment is stopped, return any unused tablets to the hospital pharmacist.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

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It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Common side effects

Diarrhoea

This treatment can cause severe diarrhoea. Diarrhoea means passing more stools (poo) than is usual for you, or having watery or loose stools. If you have a stoma, it will be more active than usual.

Your hospital team will give you anti-diarrhoea drugs to take home. It is important to take them exactly as your nurse or pharmacist explains. This means they will be more likely to work for you.

If you have diarrhoea or a mild increase in stoma activity:

- follow any advice from your cancer team about taking anti-diarrhoea drugs
- follow advice about how much fluid to drink.

Contact the hospital straight away if:

- you have diarrhoea at night
- you have diarrhoea more than 4 times in a day
- you have a moderate or severe increase in stoma activity
- the anti-diarrhoea drugs do not work within 24 hours.

You may need to go to hospital to have fluids through a drip.

Skin changes

This treatment can cause an acne-like rash on the face, neck, chest and back. The rash is most likely to start in the first 2 or 3 weeks of treatment but sometimes it can be longer. Your skin may also feel dry, itchy, scaly or tender.

Skin changes are often mild but can be more severe in some people. If you notice any skin changes, contact the hospital as soon as possible on the 24-hour number you have been given. You may need creams, steroids or antibiotics. Your doctor may stop your treatment until the skin changes improve.

Here are some things you can do to take care of your skin during treatment:

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Wash with lukewarm water and mild, unperfumed, soap-free cleansers.

- Do not use products containing alcohol on your skin, or anti-acne products (unless they have been prescribed).
- Moisturise your skin regularly with unperfumed moisturisers for dry skin conditions.
- Use sun cream of at least SPF 30 to protect your skin from the sun. Cover up with clothing and a hat.

Skin changes usually improve when treatment ends.

Sore mouth and throat

This treatment may cause a sore mouth and throat. You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Nail changes

This treatment can affect your nails. They may grow more slowly or break more easily. You might notice ridges or white or dark lines across your nails. These changes usually disappear as the nails grow out after treatment. Sometimes nails can become loose or fall off. Tell your doctor, nurse or pharmacist about any changes to your nails.

If the skin around your nails becomes sore and swollen, contact the hospital straight away on the 24-hour number. These might be signs of an infection.

Loss of appetite

This treatment can affect your appetite. Don’t worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Changes to your taste

You may get a bitter or metallic taste in your mouth. Sucking sugar-free sweets may help with this. Some foods may taste bad or have no taste. Try different foods to find out what tastes best to you. Your nurse can give you more advice.

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Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your liver is working.

Feeling sick

This treatment can cause sickness. Your doctor may give you anti-sickness drugs to help. Take the drugs exactly as your nurse or pharmacist tells you. It is easier to prevent sickness than to treat it after it has started.

If you feel sick, take small sips of fluids often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) more than once in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. Your doctor or nurse may change the anti-sickness drug to one that works better for you.

Feeling tired (fatigue)

Feeling tired is a common side effect of this treatment. Tell your cancer team if you find this difficult to manage or if it gets worse. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

You can care for your hands and feet by:

- keeping your hands and feet cool by washing in cool water
- gently moisturising your hands and feet regularly
- wearing gloves to protect your hands and nails when working in the house or garden
- wearing loose cotton socks and avoiding tight-fitting shoes and gloves.

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Hair changes

The hair on your head may become thinner when you are taking this treatment. This is usually mild. Sometimes body hair may appear thicker.

Ask your nurse for advice if you are worried about this.

Eye problems

While having dacomitinib, some people develop:

- sore, red eyes (conjunctivitis)
- itchy or dry eyes.

Tell your doctor if you develop any of these symptoms. Your doctor can prescribe eye drops to help with this.

Tell your doctor or nurse straight away if you have eyesight changes, or if your eyes become more painful or sensitive to light. Do no drive if you have these symptoms.

Less common side effects

Effects on the lungs

This treatment can cause inflammation of the lungs. This is called pneumonitis. Contact the hospital straight away on the 24-hour number if you notice any of these changes during treatment or after it ends:

- breathlessness
- a cough that does not go away
- wheezing
- a fever, with a temperature over 37.5°C.

You should also tell them if any existing breathing problems get worse. You may have:

- tests to check your lungs
- treatment, if needed.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a blood clot. Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
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reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker

- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- herbal drugs and complementary or homeopathic therapies
- recreational drugs – for example, cannabis.

Lactose

This treatment may contain lactose. If you have a lactose allergy or intolerance, ask your doctor, nurse or pharmacist for more information.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and coronavirus (covid). They may also recommend other vaccines, such as Shingrix® for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Fertility

Some cancer drugs can affect whether you can get pregnant or make someone pregnant. If you are worried about this, it is important to talk with your doctor before you start treatment.

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Sex

If you have sex during a course of this treatment, you may be advised to use barrier protection such as a condom or dental dam. This will protect your partner if any of the drug is in your semen or vaginal fluid.

Contraception

Your doctor, nurse or pharmacist will advise you not to get pregnant or make someone pregnant while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to breastfeed while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

References

Visit the electronic Medicines Compendium (eMC) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

Reviewers

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert health professionals and people living with cancer.

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Our cancer information has been awarded the PIF TICK. Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information here.

Date reviewed

Reviewed: 01 February 2024 | Next review: 01 February 2026

This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Macmillan Support Line

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The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.

Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.

Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help

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